

IMAGES IN CLINICAL MEDICINE

Floppy Eyelid Syndrome



A 39-YEAR-OLD WOMAN WITH OBESITY PRESENTED TO THE OPHTHALMOLOGY clinic with a 6-week history of foreign-body sensation and tearing in both eyes that were worse on awakening. She also reported daytime fatigue, snoring, and difficulty sleeping at night. Ophthalmologic examination showed upper-eyelid eversion with conjunctival hyperemia (Panel A) and incomplete eyelid apposition on attempted closure of both eyes (Panel B). Manual retraction of the upper eyelids while the patient was looking down showed marked laxity with complete eyelid eversion (Panel C). A diagnosis of floppy eyelid syndrome was made. Floppy eyelid syndrome is characterized by excessive laxity of the upper eyelids and results in eyelid malposition and chronic irritation of the ocular surface. The condition is associated with obstructive sleep apnea. The patient was referred for polysomnographic testing, which showed an apnea–hypopnea index of 27 events per hour, a finding consistent with a diagnosis of moderate obstructive sleep apnea. Treatment with nocturnal continuous positive airway pressure and a weight-loss program were initiated. The use of ophthalmic lubricants and eye patches while sleeping was also recommended. Two weeks after the start of treatment, the patient’s upper-eyelid eversion resolved (Panel D) and eye closure improved. In addition, her sleep quality improved and daytime drowsiness abated.

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